INFORMATION TO KEEP WITH YOU

# My complete treatment handbook

## Gemcitabine 1250 and cisplatin: nasopharyngeal cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

**Displayed calendar**
**21 days**
See the active treatment phase

**Treatment stages**
**1**
Explained in the treatment reference

**Regimen source**
**NCCP 00517**
Version 2

Day 1: both drugs; Day 8: gemcitabine. Every 21 days, 4–6 cycles.

## Treatment stages

01 **Chemotherapy course**
Day 1: both drugs; Day 8: gemcitabine. Every 21 days, 4–6 cycles.

Your team confirms the active stage and dates. The full treatment guide explains each step.

## Inside your handbook


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**My name**

**Hospital / oncology team**

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

# Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Head and neck: nasopharyngeal cancer

## When to get help

**Emergency: call 112 / 999**

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

**Contact your oncology service immediately**

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Fever, shivering, sudden illness, unusual bleeding, new breathlessness, persistent vomiting or inability to drink needs urgent oncology advice. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency: call 112/999.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

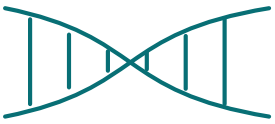
## What each medicine does

The medicines act together to shrink or control nasopharyngeal cancer. Your team records how this course fits any planned radiation treatment and your overall treatment aim.

ANTIMETABOLITE CHEMOTHERAPY

### Gemcitabine

intravenous (IV) infusion



**Interferes with deoxyribonucleic acid (the cell's genetic material) (DNA) synthesis**

Gemcitabine is converted inside cells into forms that disrupt DNA building blocks and DNA copying. It can affect normal blood-cell production as well as cancer cells.

Activated inside cells → DNA production disrupted → Cell division slowed

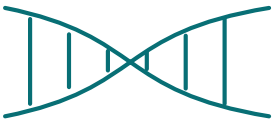
**What to watch for**

Report fever, unusual bruising, breathlessness, swelling or reduced urine urgently. Flu-like symptoms cannot safely be assumed to be a harmless drug effect.

PLATINUM CHEMOTHERAPY

### Cisplatin

Cisplatin



**Damages cancer-cell DNA**

Cisplatin links strands of DNA, the instructions inside cells, making it harder for cancer cells to repair damage and divide. Healthy cells may also be affected. Kidney function, hearing, blood counts and nerve symptoms help the team decide whether the next dose is safe.

DNA damage → Cell repair disrupted → Cancer growth reduced

**What to watch for**

Report reduced urine, persistent vomiting, tinnitus, hearing change or new tingling.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

## Understanding the evidence

The national regimen brings together research and specialist guidance for a specific treatment setting.

A small single-arm study used the same drug doses for three induction cycles before chemoradiotherapy. It does not establish a comparative survival benefit for the linked NCCP 4–6-cycle course, and it is not the 1,000 mg/m² gemcitabine randomised induction regimen.

CHAPTER 02

# Your schedule, tests and appointments

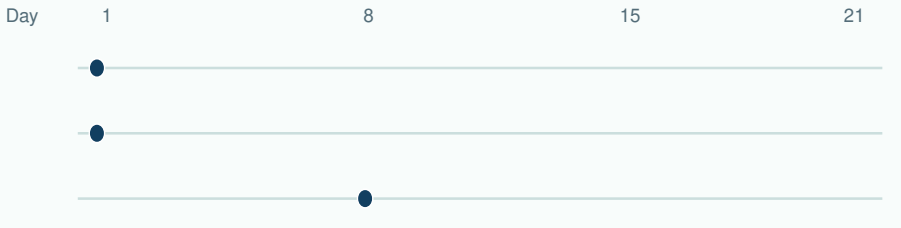
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

# Your treatment stages

**Gemcitabine 1250 and cisplatin: nasopharyngeal cancer**

21-day view




This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

DAY 1: BOTH DRUGS; DAY 8: GEMCITABINE. EVERY 21 DAYS, 4–6 CYCLES.

## Chemotherapy course

**Up to 6 cycles**

Gemcitabine 1,250 mg/m² on Days 1 and 8, with cisplatin 80 mg/m² on Day 1, every 21 days for 4–6 cycles or until progression or unacceptable side effects.

Confirm the full treatment sequence and any separate radiotherapy plan. The linked NCCP document gives 4–6 cycles; this is not the three-cycle 00903 induction schedule.

BASELINE → TREATMENT → FOLLOW-UP

# My tests and treatment checks

Routine safety tests, symptom-driven tests and response scans have different purposes.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00517 v2: retrieved official regimen

## Tests in your plan

### Full blood count

WHEN

**Before each treatment.**

Checks infection-fighting cells, oxygen-carrying cells and platelets for clotting.

### Kidney and liver tests

WHEN

**Day 1: full blood count, kidney and liver tests. Day 8: full blood count and creatinine. Hearing assessment and measured kidney clearance are arranged when indicated.**

Checks treatment handling and organ safety.

### Hearing assessment

If part of your plan

WHEN

**When clinically indicated.**

Assesses existing or new hearing changes and tinnitus.

THE WHOLE TREATMENT JOURNEY

# Gemcitabine 1250 and cisplatin: nasopharyngeal cancer: treatment and checks

**Locally advanced nasopharyngeal cancer; the full treatment strategy is agreed by the multidisciplinary team.**

Gemcitabine 1,250 mg/m² on Days 1 and 8, with cisplatin 80 mg/m² on Day 1, every 21 days for 4–6 cycles or until progression or unacceptable side effects.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | Treatment visits | Between visits | Next cycle / phase | After treatment |
|---|---|---|---|---|---|
| **Blood and organ checks** NCCP | Full blood count, kidney and liver tests; review medicines, allergies, weight, oral intake and symptoms. Review nerve symptoms and hearing; audiology when indicated. Cisplatin eligibility includes adequate kidney function. | Day 1: full blood count, kidney and liver tests. Day 8: full blood count and creatinine. Hearing assessment and measured kidney clearance are arranged when indicated. | Extra tests may be needed for symptoms or abnormal results. | Review results, recovery and earlier toxicity before another dose. | The team follows recovery, response and persistent side effects. Scans and tumour tests follow the individual plan; there is no universal scan or tumour-marker schedule in this regimen. |

| Care / check | Before starting | Treatment visits | Between visits | Next cycle / phase | After treatment |
|---|---|---|---|---|---|
| **Treatment and appointments**<br>NCCP | Confirm the complete treatment plan. | Gemcitabine runs over 30 minutes, followed on Day 1 by cisplatin over 2 hours. Cisplatin requires prescribed fluids before and after treatment. Day 8 is gemcitabine alone. | Follow written medicine and contact instructions. | Confirm the full treatment sequence and any separate radiotherapy plan. The linked NCCP document gives 4–6 cycles; this is not the three-cycle 00903 induction schedule. | Record completion and the named follow-up team. |
| **Mouth, weight and swallowing**<br>NCCP | Assess symptoms, teeth, eating and drinking. | Review weight, intake and mouth symptoms. | Call for difficulty swallowing, worsening pain or weight loss. | Dietitian and swallowing support according to need. | Continue rehabilitation and dental care if advised. |

## What each check is for

1

### Treatment course

Day 1: both drugs; Day 8: gemcitabine. Every 21 days, 4–6 cycles.

The medicines act together to shrink or control nasopharyngeal cancer. Your team records how this course fits any planned radiation treatment and your overall treatment aim.

**Ask / act:** Confirm active medicines, safety checks and support plan.

2

### Review / next phase

After the planned course and recovery review

Review response and recovery.

**Ask / act:** Confirm actual dates and the next prescription.

**At the next review, ask**

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

## Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

### Before your first visit

Full blood count, kidney and liver tests; review medicines, allergies, weight, oral intake and symptoms. Review nerve symptoms and hearing; audiology when indicated. Cisplatin eligibility includes adequate kidney function.

STEP 2

### Time in the unit

Gemcitabine runs over 30 minutes, followed on Day 1 by cisplatin over 2 hours. Cisplatin requires prescribed fluids before and after treatment. Day 8 is gemcitabine alone.

STEP 4

### Leaving the unit

Confirm the next tests, medicines, treatment dates and verified daytime and out-of-hours contact numbers.

STEP 5

### Practical help

Ask about transport, work, costs, communication needs and whether a support person can attend.

## Your exact treatment schedule

## Tests and preparation

**During treatment**

Day 1: full blood count, kidney and liver tests. Day 8: full blood count and creatinine. Hearing assessment and measured kidney clearance are arranged when indicated.

CHAPTER 03

# Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

## Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

| Treatment or check | Timing in this view |
|---|---|
| **Gemcitabine** |  **Day 1** Gemcitabine 1250 mg/m² on Day 1. |
| **Cisplatin + fluids** |  **Day 1** Cisplatin 80 mg/m² on Day 1, with prescribed hydration. |
| **Gemcitabine** |  **Day 8** Gemcitabine 1250 mg/m² on Day 8 after safety review. |
| **Recovery and review** |  **Days 9–21** Follow the support plan and report symptoms promptly. |

**Prescribed timing:** Gemcitabine 1,250 mg/m² on Days 1 and 8, with cisplatin 80 mg/m² on Day 1, every 21 days for 4–6 cycles or until progression or unacceptable side effects.

**Safety throughout:** Symptoms and urgent-contact advice apply throughout treatment and after the last dose.

**Review and next step:** Confirm the full treatment sequence and any separate radiotherapy plan. The linked NCCP document gives 4–6 cycles; this is not the three-cycle 00903 induction schedule.

> **Why these bands stay broad:** This is a reference cycle, not a personal appointment calendar. Side effects can occur on any day; the recovery interval is not a guarantee of safety. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00517 v2: retrieved official regimen ↗

> **There are no guaranteed "safe days".** Use your treating service's written temperature threshold and urgent-contact plan. Fever, shivering or sudden illness can be serious even without a high temperature. Do not wait for the next visit. Severe chest pain, severe breathing difficulty, collapse or a seizure needs emergency help: call 112 or 999.

**Between-treatment notes**

### Day 1: Day 1: treatment

Gemcitabine runs over 30 minutes, followed on Day 1 by cisplatin over 2 hours. Cisplatin requires prescribed fluids before and after treatment. Day 8 is gemcitabine alone. Report new symptoms before dosing.

### Day 2–7: Between the first two visits

Follow prescribed support medicines. Blood counts and side effects may change between visits; this is not a guaranteed safe period. Call the team promptly if unwell. Keep the Day-8 blood-test and treatment appointment.

### Day 8: Day 8: gemcitabine

Day 1: full blood count, kidney and liver tests. Day 8: full blood count and creatinine. Hearing assessment and measured kidney clearance are arranged when indicated. Day-8 decisions use their own blood-count and toxicity rules.

### Day 9–21: Recovery and next review

Keep the next test and treatment dates. Fatigue and low blood counts may occur between visits. Do not wait for the next visit if unwell.

## Support around the medicines

**1** **Preventing and treating sickness**

Use the prescribed medicines for the active treatment day. The cisplatin combination visit has high sickness risk; Day 8 is gemcitabine alone and has a separate plan.

**2** **Fluids with cisplatin**

Cisplatin needs prescribed fluids before and after treatment. Follow your individual drinking plan for the next 24 hours; heart or kidney conditions may change it. Call if vomiting prevents drinking or urine falls.

**3** **Mouth care and nutrition**

Follow your unit’s oral-care plan. Tell the team early about mouth pain, weight loss, choking or difficulty eating and drinking. Dietetic and swallowing support can help.

**4** **Medicine review**

Bring prescription, non-prescription and herbal medicines. Check interactions, allergies and any changes with the oncology pharmacist.

**5** **Recovery and practical support**

Ask about fatigue, work, transport, caring responsibilities, emotional support and help with appointments.

## Understanding your diagnosis

### Tumour tests and safety tests

Your team explains cancer-specific tests, including Epstein–Barr virus tests where relevant in nasopharyngeal cancer. These are distinct from blood tests used for medicine safety. No universal tumour-marker schedule is prescribed here.

## Your treatment aim and choices

### Deciding together

Discuss expected benefit, possible lasting effects, suitable alternatives and the option of declining treatment. Tell the team what matters most to you.

## Symptoms and when to call

### Between visits

Keep a record of symptoms and medicines taken. Do not wait for a routine appointment when symptoms are worsening.

## Mouth, eating and swallowing

### Speak up early

Tell the team about pain, dry mouth, taste changes, choking, reduced intake or weight loss. Ask for a personalised oral-care, dietetic and swallowing plan.



### Radiation-related care

If you are having head and neck radiation, follow the radiation team’s dental, skin, saliva and swallowing advice during and after treatment. Radiation appointments and late-effect checks have their own plan.

## Living with treatment

### Activity and rest

Pace activity according to symptoms. Ask for help with fatigue, sleep, work, travel or caring responsibilities.

### Sexual health and fertility

Discuss fertility preservation before treatment where appropriate. Ask about contraception, pregnancy, breastfeeding and medicine-specific timeframes.

## Infusions and access

### Your infusion arrangement

### Line problems

Report pain, redness, swelling, leaking or a wet dressing. Do not adjust infusion equipment yourself; contact the unit.

## After this course

### Recovery and follow-up

The team follows recovery, response and persistent side effects. Scans and tumour tests follow the individual plan; there is no universal scan or tumour-marker schedule in this regimen.

## Questions for your team

### My plan

What is the treatment aim? Which tests are mandatory? What happens if a treatment day is missed?

### My support

Which medicines do I take and when do I stop them? Whom do I contact overnight?

## Questions worth asking

**undefined**

undefined

**undefined**

undefined

**undefined**

undefined

**undefined**

undefined

**undefined**

undefined

**undefined**

undefined

**undefined**

undefined

**Try explaining the plan back:** Can you explain your treatment days, required blood tests, support medicines and whom to call if unwell? Ask the team to write down any unclear part.

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Day 1: combination

**Active medicines:** Gemcitabine + Cisplatin

**Emetogenic risk:** high

| Medicine | Dose / route | When |
|---|---|---|
| Aprepitant | 125 mg PO OD | Day 1<br>Clinician/pharmacy draft; reconcile to the individual prescription. |
| Ondansetron | 16 mg PO OD | Day 1<br>Clinician/pharmacy draft; reconcile to the individual prescription. |
| Dexamethasone | 12 mg PO OD | Day 1<br>Clinician/pharmacy draft; reconcile to the individual prescription. |
| Olanzapine | 5 mg PO OD | Day 1<br>Clinician/pharmacy draft; reconcile to the individual prescription. |
| Aprepitant | 80 mg PO OD | Days 2-3<br>Clinician/pharmacy draft; reconcile to the individual prescription. |
| Dexamethasone | 8 mg PO OD | Days 2-4<br>Clinician/pharmacy draft; reconcile to the individual prescription. |
| Metoclopramide | 10 mg PO TDS | Subsequent days / as directed<br>Clinician/pharmacy draft; reconcile to the individual prescription. |
| Olanzapine | 5 mg PO OD | Days 2-4<br>Clinician/pharmacy draft; reconcile to the individual prescription. |

### Additional phase-specific support

- Gemcitabine runs over 30 minutes, followed on Day 1 by cisplatin over 2 hours. Cisplatin requires prescribed fluids before and after treatment. Day 8 is gemcitabine alone.
- Day 1: full blood count, kidney and liver tests. Day 8: full blood count and creatinine. Hearing assessment and measured kidney clearance are arranged when indicated.
- Confirm the full treatment sequence and any separate radiotherapy plan. The linked NCCP document gives 4–6 cycles; this is not the three-cycle 00903 induction schedule.

- The local CUH source is retained as a reference list, not as the national antiemetic standard. Nystatin/chlorhexidine are displayed as local mouth-care adjuncts, not universal antiemetic or mucositis prophylaxis. Verify current local oral-care policy.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

## Day 8: gemcitabine only

**Active medicines:** Gemcitabine

**Emetogenic risk:** low

| Medicine | Dose / route | When |
|---|---|---|
| Ondansetron | 8 mg PO OD | Day 1<br>Clinician/pharmacy draft; reconcile to the individual prescription. |
| Metoclopramide | 10 mg PO TDS | PRN<br>Clinician/pharmacy draft; reconcile to the individual prescription. |

### Additional phase-specific support

- Gemcitabine runs over 30 minutes, followed on Day 1 by cisplatin over 2 hours. Cisplatin requires prescribed fluids before and after treatment. Day 8 is gemcitabine alone.
- Day 1: full blood count, kidney and liver tests. Day 8: full blood count and creatinine. Hearing assessment and measured kidney clearance are arranged when indicated.
- Confirm the full treatment sequence and any separate radiotherapy plan. The linked NCCP document gives 4–6 cycles; this is not the three-cycle 00903 induction schedule.
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**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

# What to watch for

You do not need to know which medicine caused a symptom before calling.

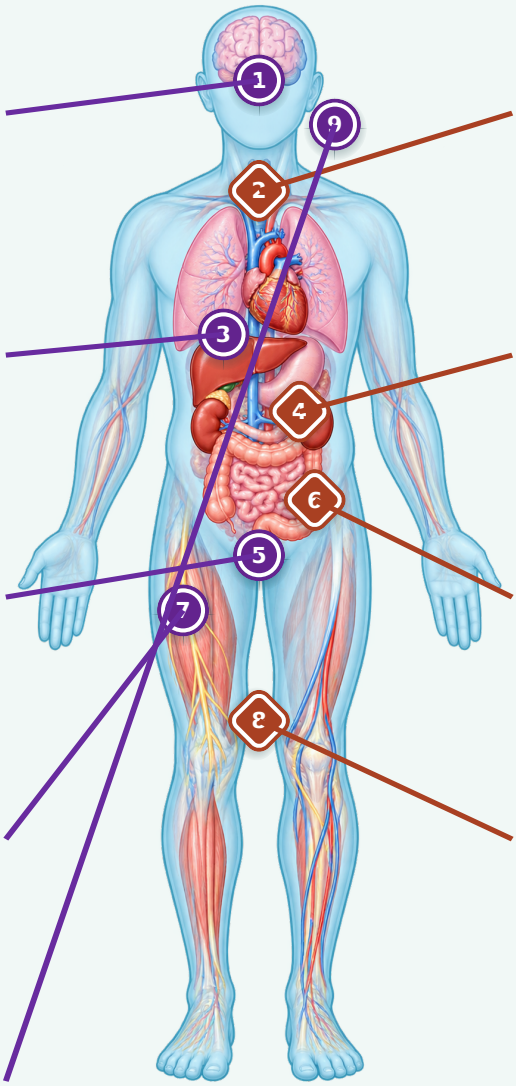
### 1 Fatigue and concentration
**Gemcitabine + Cisplatin**
Tiredness can affect activity and concentration. Balance activity and rest; report sudden confusion.

### 3 Breathing
**Gemcitabine + Cisplatin**
New or worsening breathlessness, cough or chest symptoms need urgent advice; lung inflammation can be serious.

### 5 Sickness and bowels
**Gemcitabine + Cisplatin**
Take prescribed anti-sickness medicines. Report persistent vomiting, diarrhoea, dehydration or inability to drink.

### 7 Skin and infusion reactions
**Gemcitabine + Cisplatin**
Report severe rash, blistering, facial swelling or breathing symptoms promptly. Tell staff about pain at the infusion site.

### 9 Hearing and nerves
**Cisplatin**
Report ringing, hearing loss, numbness or tingling before the next dose; some effects can persist.




### 2 Mouth and swallowing
**Gemcitabine + Cisplatin**
Report soreness, ulcers or difficulty swallowing. Ask early about mouth care, pain relief and nutrition.

### 4 Blood and infection
**Gemcitabine + Cisplatin**
Fever, shivering, sudden illness, unexplained bruising or bleeding needs urgent oncology advice.

### 6 Kidneys and fluid balance
**Gemcitabine + Cisplatin**
Report reduced urine, swelling or poor fluid intake. Blood tests help detect kidney injury.

### 8 Fertility and pregnancy
**Gemcitabine + Cisplatin**
Discuss fertility preservation, contraception, pregnancy and breastfeeding before treatment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivering, sudden illness, unusual bleeding, new breathlessness, persistent vomiting or inability to drink needs urgent oncology advice. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency: call 112/999.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

CHAPTER 04

# Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Record actual treatment dates and dose changes. Your clinician confirms the course length and any later phase.

· copy this sheet for another cycle of the same phase.

## Gemcitabine · Antimetabolite chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |
| **Bleeding / bruising** | □ None | □ New small bruise / brief bleed: report | □ Repeated bleeding: call today | □ Bleeding that will not stop: urgent help | □ Heavy bleeding / collapse: 112/999 |
| **Nausea** | □ None | □ Queasy but eating and drinking | □ Intake reduced: contact the team | □ Unable to maintain intake: urgent review | □ Collapse / severe dehydration: 112/999 |

## Cisplatin · Platinum chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Nausea** | □ None | □ Queasy but eating and drinking | □ Intake reduced: contact the team | □ Unable to maintain intake: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |
| **Urinary symptoms** | □ Usual pattern | □ Mild change: report | □ Pain or frequent urination: call | □ Cannot pass urine / marked bleeding: urgent review | □ Collapse or severe illness: 112/999 |
| **Hearing changes** | □ No change | □ New ringing or muffled hearing: report promptly | □ Conversation difficult: contact the team | □ Sudden hearing loss: urgent assessment | □ Sudden weakness or collapse: 112/999 |
| **Numbness / tingling** | □ None | □ Mild change; usual tasks unaffected | □ Limits buttons, writing or walking: call | □ Washing, dressing or walking difficult: urgent review | □ Sudden major weakness / paralysis: emergency help |
| **Fever / chills** | □ No fever or chills | □ New chills / feeling unwell: call | □ Fever: use the written urgent plan now | □ Fever with marked weakness: urgent assessment | □ Collapse / confusion / breathing difficulty: 112/999 |
| **Bleeding / bruising** | □ None | □ New small bruise / brief bleed: report | □ Repeated bleeding: call today | □ Bleeding that will not stop: urgent help | □ Heavy bleeding / collapse: 112/999 |
| **Infusion symptoms** | □ None | □ New flushing, chills or itching: tell the nurse immediately | □ Symptoms during infusion: tell the nurse immediately | □ Wheeze, swelling or faintness: immediate urgent help | □ Severe breathing difficulty or collapse: emergency help |

Dates / doses missed or changed / symptoms affecting daily life / questions:

> Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

# Keep your treatment details to hand

**Head and neck: nasopharyngeal cancer** · NCCP 00517 v2 · Document revised 11 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

**FRONT / CONTACTS**

## My Treatment Card

SACTCheck

Name: _____

**Gemcitabine 1250 + cisplatin**

Day 1: both drugs; Day 8: gemcitabine. Every 21 days, 4–6 cycles.

Phase: _____ Planned cycles: 6

**Daytime (EXAMPLE)** 087 12345678

**Out of hours (EXAMPLE)** 087 87654321

**Unwell or fever? Call oncology now.**
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

**BACK / TREATMENT RECORD**

## My cycle record

Team stamps after each completed cycle.

| C1 | C2 | C3 |
|---|---|---|
| Date / initials | Date / initials | Date / initials |
| **C4** | **C5** | **C6** |
| Date / initials | Date / initials | Date / initials |

My treatment information

Plan changed? Ask the team to update this card.

CUT outer border

**FOLD centre — do not cut**

## Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

## Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

### iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

### Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

## Your local contacts and resources

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.49 · NCCP 00517 v2 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

CHAPTER 05

# Sources and further information

## IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.


1. **NCCP 00517 v2 · pTreatment / supportive care**
   https://healthservice.hse.ie/documents/6897/517_V2_GemCis80.pdf
2. **NCI: head and neck cancers**
   https://www.cancer.gov/types/head-and-neck/head-neck-fact-sheet
3. **NCI: oral complications of cancer therapies, patient version**
   https://www.cancer.gov/about-cancer/treatment/side-effects/mouth-throat/oral-complications-pdq
4. **NCI: nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
5. **HSE: cancer treatment side effects**
   https://www.hse.ie/conditions/cancer/if-you-have-side-effects/
6. **NCCP supportive-care antiemetics for NCIS**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
7. **Yau et al., Head Neck 2006**
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